

HEART DISEASE & STROKE

Coronary heart disease is the main form of heart disease. It occurs when the coronary arteries, which supply blood to the heart muscle, become hardened and narrowed due to a buildup of plaque on the arteries' inner walls. Plaque is the accumulation of cholesterol, fat, and other substances. As plaque continues to build up in the arteries, blood flow to the heart is reduced.

Coronary heart disease—often simply called heart disease—can lead to a heart attack. A heart attack happens when a cholesterol-rich plaque bursts and releases its contents into the bloodstream. This causes a blood clot to form over the plaque, totally blocking blood flow through the artery and preventing vital oxygen and nutrients from getting to the heart. A heart attack can cause permanent damage to the heart muscle.

Heart disease includes a number of other serious conditions, including:

Angina is chest pain or discomfort that occurs when the heart muscle is not getting enough blood. The inadequate blood flow is caused by narrowed coronary arteries, due to an accumulation of plaque. A bout of angina is not a heart attack, but it means that you're more likely to have a heart attack than someone who doesn't have angina. There are two kinds of angina:

Stable angina has a recognizable pattern. It may feel like pressure or a squeezing pain in your chest. The pain may also occur in your shoulders, arms, neck, jaw, or back. It may also feel like indigestion. Stable angina pain is generally brought on by some kind of exertion or strain (such as climbing stairs or experiencing emotional stress), and it is usually relieved by rest or medicine.

Unstable angina is more serious than stable angina. Occurring at any time, unstable angina often reflects a change in a previously stable pattern of angina. Episodes of unstable angina are usually more frequent, painful, and longer lasting than bouts of stable angina, and are less often relieved by rest or medicine. Unstable angina is a sign that you may have a heart attack very soon. The symptoms are the same as if you are having a heart attack.

Congestive heart failure. Congestive heart failure is a life-threatening condition in which the heart cannot pump enough blood to supply the body's needs. Heart failure occurs when excess fluid collects in the body as a result of heart weakness or injury. This condition leads to a buildup of fluid in the lungs, causing swelling of the feet, tiredness, weakness, and breathing difficulties.

There are several important things a caregiver can do to help a loved one with heart failure to make day-to-day living comfortable, to avoid acute episodes of worsening heart failure, and to improve their long-term outcome. These include:

- helping the person with heart failure establish and maintain lifestyle changes that will benefit their heart
- helping them keep up with what is most often a complicated medication schedule
- helping to monitor the daily measurements and the symptoms that might indicate that the heart failure is worsening
- recognizing when it is time to call the doctor or go to the hospital

1. Medication

People with heart failure usually take a lot of medication, and it is critical to keep track of the pills they are taking. It is usually best for caregivers to let the person with heart failure be the primary manager of their own medication, but it's often a good idea to double check. Using pill boxes or a chart makes it easier for everyone to know what has been taken and what has been missed.

- Patients should be safe with medicines by taking exactly as prescribed. You should call the Doctor if you think a patient is having a problem with medicine.
- They should not take any vitamins, over-the-counter medicine, or natural health products without talking to the doctor first.
- They shouldn't take ibuprofen (Advil or Motrin) and naproxen (Aleve) without talking to the doctor first. They could make heart failure worse.

Some of the recommended medicine are:

- Angiotensin-converting enzyme inhibitors (ACEIs) or angiotensin II receptor blockers (ARBs) reduce the heart's workload, lower blood pressure, and reduce swelling. Taking an ACEI or ARB may lower your chance of needing to be hospitalized.

- Beta-blockers can slow heart rate, decrease blood pressure, and improve your condition. Taking a beta-blocker may lower your chance of needing to be hospitalized.
- Diuretics, also called water pills, reduce swelling.

2. Checking weight

The patient will get to know what weight is right for them. Weighing them often will help you know if there is too much fluid in their body. You might also find that their clothes and shoes are feeling tighter than normal when there is too much fluid in the body.

Try and weigh a patient every morning on the same scale when they get up -- before they eat and after using the bathroom. Make sure they are wearing similar clothing each time you weigh them. Write down their weight every day on a chart so that you can keep track of it.

Call the doctor if they have a sudden weight gain, such as more than 1 kg (2 lb) to 1.3 kg (3 lb) in a day or 2.3 kg (5 lb) in a week. The doctor may suggest a different range of weight gain. A sudden weight gain may mean that heart condition is getting worse.

3. Checking heart rate and pulse

The radial pulse is felt on the wrist, just under the thumb



Know what your patient's normal pulse rate is. Your provider will tell you what it should be.

You can take your pulse in the wrist area below the base of your thumb. Use your index and third fingers of your other hand to find the pulse. Count the number of beats for 30 seconds. Then double that number. That is the pulse.

4. Checking blood pressure



The medical provider may ask you to keep track of the patient's blood pressure at home. Make sure you get a good quality, well-fitting home device. Show it to your provider or nurse. It will probably have a cuff with a stethoscope or a digital readout. Make sure to get assistance from the doctor depending on the time of device you're using.

5. Activity level

- Start light exercises with the patient if the doctor says it is okay. Even if they can only do a small amount, exercise will help them get stronger, have more energy, and manage weight and stress. Walking is an easy way to get exercise. Start out by walking a little more than they did before. Bit by bit, increase the amount you walk.
- When you exercise with the patient, watch for signs the heart is working too hard. You are pushing yourself too hard if they cannot talk while they are exercising. If they become short of breath or dizzy or have chest pain, stop, sit down, and rest.

- If they feel "wiped out" the day after exercising, walk slower or for a shorter distance until they can work up to a better pace.
- They should get enough rest at night. Sleeping with 1 or 2 pillows under their upper body and head may help breathe easier.

6. Lifestyle changes.

- Do not smoke. Smoking can make a heart condition worse. If they need help quitting, talk to the doctor about stop-smoking programs and medicines. These can increase chances of quitting for good. Quitting smoking may be the most important step to take to protect the heart.
- Limit alcohol. Too much alcohol can cause health problems.
- Avoid getting sick from colds and influenza (flu). The patient should get a pneumococcal vaccine shot. If they have had one before, ask the doctor whether they need a second dose. Get a flu shot each year. If they must be around people with colds or the flu, encourage them to wash hands often.

Stroke

The care of patients with stroke begins in the hospital and continues in the community, where recovery, reintegration, and health maintenance take place over the years that follow. Primary care clinicians provide most of this long-term care. The needs of these patients can be complex according to the temporal phase of their illness, the cause and severity of their stroke, and other factors, including the presence of other chronic health conditions.⁷ Approximately 50% to 80% will have hypertension, 20% to 30% will have diabetes, and 10% to 30% will have comorbid heart disease or atrial fibrillation. Lung disease, depression, anxiety, kidney disease, and arthritis are also common.